

Patient 01**Report A**

6485298 04/05/2016 CT Acute Stoke

For Thrombolysis? No, outside treatment time window

Report:

Indication: Expressive aphasia. Mild right-sided weakness. Ex-smoker. 2 Left cortical stroke. ? Carotid disease.

CT HEAD Haemorrhage: No Intravascular thrombus: No

Acute infarct: Yes. There is diminished grey-white matter differentiation involving the left precentral gyrus, in keeping with a subacute infarct within the territory of the left middle cerebral artery with further low attenuation suspicious for infarction demonstrated in the left insula. Two small mature infarcts are noted in the left cerebellar hemisphere. Other findings and comments: There is some thickening of the right maxillary sinus mucosa featuring a probable mucous retention cyst.

CT ANGIOGRAM

Aortic arch and subclavian arteries: No significant aortic arch atheroma is demonstrated.

RCCA/ICA: Movement/pulsation artefact degrades a short segment of the right proximal common carotid arteries, but the remainder of the vessel opacifies normally. There is no significant right carotid bifurcation/proximal ICA atheroma.

LCCA/ICA: Some artefact as on the right. There is no significant carotid bifurcation/proximal ICA atheroma.

RVA: Non-dominant, with some non-stenosing calcific atheroma at the origin of the right vertebral artery. The vessel opacifies normally.

LVA: Minimal atheroma at the origin.

Intracranial Arteries: No proximal stenosis or vessel occlusion is demonstrated. Other findings and comments: Incidental note made of incomplete fusion of the anterior and posterior arch of C1 in the midline (anatomical variant). There is scarring at the right lung apex. A coarse focus of calcification is shown caudal to the coracoid process of the shoulder, which may relate to degenerative change or previous trauma. Conclusion: Evolving left MCA costical infarct.

Report B

6485326 05/05/2016 MR Head

6485326 05/05/2016 MR MRA

Clinical information: Expressive aphasia, dyspraxic and slightly weak right arm ? infarct

Findings:

Comparison made with the CT head and carotid/cerebral angiogram dated 04/05/2016

There is predominantly cortically-based diffusion restriction involving the left precentral gyrus and superior frontal gyrus in keeping with an acute MCA territory infarct. Further recent infarcts are shown in the left parietal lobe and involving the dorsal aspect of the left insula with less avid diffusion signal abnormality, which would raise the possibility of staggered onset. There is no significant haemorrhagic transformation. Tiny foci of susceptibility artefact are shown in the right parietal and left temporal lobe respectively, likely to represent microhaemorrhages. A few additional T2/FLAIR hyperintensities are present in the cerebral white matter bilaterally, most likely representing microangiopathic ischaemic changes.

The pituitary gland appears bulky for the patient's age, although no discrete lesion is visualised. Correlation with hormonal profiling is suggested in the first instance, and additional targeted pituitary MRI sequences could be helpful to exclude an adenoma. The remaining intracranial appearances are normal. Normal flow is demonstrated in the main intracranial arteries.

Comments: Recent left MCA infarction. Bulky pituitary gland.

